

60%

parents ignore signs of stress and tend to consult doctors for falling grades

DEATHS

Psychologists call it the cluster effect. And Uttar Pradesh is a case in point. On March 31, Priya Bose, 13, hanged herself over exam stress. It sparked off a spree. In the last two weeks, nearly 10 students chose to snuff out their lives. Experts call it copycat deaths. "Children see it as a glamorous way to die, a way to get a lot of attention that they don't get in life," says psychiatrist, Rajesh



CLUSTER: Eight teen suicides in the last two weeks in Lucknow

Sharma, of AIIMS. "We are witnessing a new trend of suicide contagion," rues Carlyle McFalin, head of English at Lucknow's Christ Church College. Psychiatrist S.C. Tewari of King George Medical University, Lucknow, too, finds herd mentality at work: "Students from posh schools are obsessed with success. A failure can push them over the edge." He treats a lot of youngsters who are on performance-enhancing drugs. With all this, the state is unnerved: Grills along the Gomati river, exam boards holding back merit lists or doing away with irrelevant courses. But with student suicide rates climbing six-fold in five years, will that be enough to put out the contagion effect? *Subhash Mishra*

prevalent among affluent students of public schools," says Om Prakash Sharma, MLC and president of the Uttar Pradesh Madhyamik Shikshak Sangh.

Although suicide is a deeply personal and individual act, it is determined by a range of factors, say experts. Shekhar Sheshadri of NIMHANS says, "Suicide is perceived as a social problem in our country and hence, mental disorder is given equal conceptual status with family conflicts, social maladjustment, etc." Now, new research on teen brains highlights the role of biology behind it. Brain scans of adolescents show that the area of sober second thoughts—prefrontal cortex—matures slowly, explains Dr Shamsah Sonawalla, consultant psychiatrist with Jaslok Hospital, Mumbai and Harvard Medical School. The emotional centre, in charge of primal fears and anxiety, is fully active. "Hence, quite a bit of adolescent decision-making is not reasoned out but impulsive," she adds. Under the influence of raging hormones, youngsters often don't quite understand the finality of their action. "I don't think they think about being dead. They think it's a way of ending pain and solving a problem," she adds.

Growing pains were always tended to within the family. But for many teenagers a traditional family structure no longer exists, and divorce, separation or remarriage of parents have created new pressures. "Faster-paced lives with parents absorbed in their own lives and careers, often provoke feelings of alienation, rejection and emotions that can lead teenagers to suicide," says Dr Rajesh Sagar of AIIMS. Against this background, psychologists hold, young people seem to be taking setbacks

harder than the generations before them did, turning to suicide as the last step in an accelerated process of breakdown. In a research, Dr Mridula Kayal of AIIMS studied 33 adolescents with depressive disorders. Nearly 90 per cent of those who responded well to treatment were found to have supportive families. In cases that showed dramatic improvement, parental attention on the child averaged four hours. And the majority of depressed teens had pushy parents who wanted to control every aspect of their lives.

Strangely enough, in most cases,

family members remain unaware of the inchoate emotional forces running rampant in adolescent minds. A NIMHANS study revealed 65 per cent families do not have an inkling about their children having depression or suicidal tendencies. Sheshadri says, "This is one of the major reasons why suicide rates are so high. Parents often don't pick up the signs of disorders where the child internalises

problems and gets bogged down by anxiety, phobias, academic and socialisation plights." Any of these may cause a susceptible person to break down and slide into depression. "Unfortunately, over 60 per cent parents ignore other signs of stress—irritability, temper tantrums, sudden withdrawal, suicidal dreams—and come to doctors for falling school performance," adds Sagar. Consider 16-year-old Tanvi, who shot herself with her father's revolver this March. She wasn't doing well in school and just two days before the results her teachers had summoned her parents. Nobody had understood the extent of her anxiety. "She seemed so normal the previous night at the family din-

"It's not that they think about being dead. To most youngsters, it's a way of ending pain and solving a problem."

SHAMSAH SONAWALLA
PSYCHIATRIST, JASLOK & HARVARD